Op. Dr. Murat Ayata
Kulak Burun Boğaz
Otolaryngology Head and Neck Surgery Specialist

Op. Dr. Murat Ayata Private Clinic
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

INFORMED CONSENT FORM

Otoplasty

Surgery to correct prominent ears (ear pinning)

Dear patient, this form has been prepared to explain, in **plain and clear language**, what you need to know about the procedure recommended to you. Its purpose is not to frighten you, but to help you make your decision **freely and on the basis of good information**.

Please read this form without rushing. Ask your doctor about anything you do not understand. Your doctor will also explain verbally any risks that are particular to your situation. Whether or not you give consent is entirely your right.

PATIENT AND PROCEDURE DETAILS

Full name: _____ Date of birth: _____

Passport / ID No: _____ File / Protocol No: _____

Telephone: _____ Date informed: _____

Planned procedure date: _____

Form first sent to patient (date / e-mail, WhatsApp, in person): _____

Doctor performing the procedure and providing information: **Op. Dr. Murat Ayata**

Hospital / facility where the procedure will be performed: _____

Side: ☐ Right ☐ Left ☐ Both

Type of anaesthesia: ☐ General anaesthesia ☐ Local anaesthesia ☐ Local anaesthesia + sedation

Parts to be corrected / patient's expectations: _____

1 Your condition and the aim of the procedure

Prominent ears are ears that stand out from the head at a wider angle than usual, or in which the natural fold of the ear cartilage (the antihelix) has not developed. The condition is usually present from birth, is not harmful to health and does not affect hearing. However, especially in childhood and adolescence, it can lead to teasing, loss of self-confidence and social anxiety.

The aim of otoplasty is to bring the ears closer to the head so that they look **natural** and as **balanced** as possible on both sides. Because the ear reaches most of its adult size by around 5–6 years of age, the operation can be performed **from the age of 5–6**; doing it before school age may prevent the child from being affected socially. If your child is to have the operation, it is important that their own wishes and readiness are taken into account.

2 How is the procedure performed?

The operation is performed in an operating theatre. General anaesthesia is usually used for children; for adults, local anaesthesia (with sedation if needed) or general anaesthesia is used.

1. **Planning and marking:** The ears are photographed, their distance from the head is measured, and the position of the new fold is marked.
2. **Incision:** An incision is made in the skin on the **back of the ear**, close to the natural crease between the ear and the head. As this scar lies behind the ear, it is usually not visible.

Op. Dr. Murat Ayata Private Clinic
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

3. **Shaping the cartilage:** A new fold (antihelix) is created in the ear cartilage with permanent stitches. If necessary, the cartilage is thinned or lightly scored to soften it.
4. **Setting the ear back:** If the deep bowl of the ear (the concha) is large, it is reduced or fixed with stitches to the tissue behind the ear. If the earlobe also sticks out, it is corrected.
5. **Closure and dressing:** The skin is closed with stitches. A soft, lightly compressive head dressing is applied to hold the new shape of the ears and prevent blood from collecting.

ANAESTHESIA	APPROXIMATE DURATION	HOSPITAL STAY / DISCHARGE
General anaesthesia in children; local anaesthesia ± sedation or general anaesthesia in adults	About 1–2 hours (both sides)	Usually discharged the same day

3 Expected benefits and likelihood of success

- Ears that sit closer to the head and look natural
- A more balanced appearance between the two ears
- Less social anxiety and teasing related to appearance; a boost to self-confidence

Most patients and families are satisfied with the result. Because ear cartilage is a flexible tissue, published studies report that in about **5–10 out of 100** patients the ear may partly spring forward again and a correction may be needed. Differences of a few millimetres between the two ears are considered normal.

4 Other treatment options

- **No surgery:** Prominent ears do not cause health problems, so surgery is not essential.
- **Moulding (splinting) in the newborn period:** In the first few weeks after birth the ear cartilage is very soft and can be shaped with special moulds; this is not effective after that period.
- **Camouflage with a hairstyle:** Hairstyles that cover the ears can hide their appearance.
- **Incisionless / thread techniques:** In selected patients with soft cartilage, correction can be done through tiny incisions; the recurrence rate may be higher than with standard surgery.

5 What happens if the procedure is not performed?

If the operation is not performed, the shape of the ears will stay the same; it does not improve by itself over time. Prominent ears pose no danger to health; the decision to operate depends entirely on appearance and on the wishes of the person (for children, the child and family).

You have the right to decline the recommended procedure, to postpone it, or to seek a second medical opinion. This decision will not affect any other healthcare offered to you in any way. You may withdraw your consent at any time before the procedure begins; please be aware, however, that once a procedure has started there may be stages at which stopping would be medically unsafe.

6 Preparing for the procedure

- The necessary blood tests are done before surgery; if general anaesthesia is planned, you/your child will be assessed by the anaesthetist.
- If general anaesthesia or sedation is used, nothing should be eaten or drunk for **6–8 hours** before the operation (for young children, the time stated by the anaesthetist).
- Wash the hair with shampoo on the morning of surgery. Long hair can be tied back away from the operation area.
- Surgery may be postponed if there is an active skin infection, eczema or an unhealed piercing around the ear.
- Tell your doctor about **all medicines** you take (including blood thinners, aspirin, painkillers, weight-loss and hormone medicines) and any **herbal products and supplements** (such as omega-3, vitamin E, ginkgo, garlic, ginseng, fish oil). Do not stop any medicine on your own without your doctor's advice.

Op. Dr. Murat Ayata Private Clinic
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

- Be sure to tell us about any **allergies**, bleeding or clotting problems, heart, lung, diabetes, blood pressure or thyroid conditions, previous operations and any problems you have had with anaesthesia.
- If there is any chance you may be pregnant, or if you are breastfeeding, tell us before the procedure.
- Smoking and nicotine products significantly impair wound healing. If possible, stop at least 2–4 weeks before the procedure and do not smoke during recovery.

7 Recovery: what to expect

First 48 hours	Throbbing, tightness and moderate pain in the ears may occur and are controlled with painkillers. Sleeping with the head raised is recommended. **Severe pain that keeps increasing on one side is not normal** and must be reported immediately.
Days 2–7	The dressing is usually removed at a follow-up visit between **days 2 and 5** and the ears are checked. Swelling, bruising and tenderness of the ears are common at this stage. After the dressing is removed, a soft **headband (ear band)** is worn. Children can usually return to school after one week.
Weeks 1–2	The headband is worn **day and night for the first 1–2 weeks**, then **at night only for 4–6 weeks**. This prevents the ear from folding forward during sleep and protects the stitches. Non-dissolvable skin stitches, if used, are removed on days 7–10.
Weeks 3–6	Most of the swelling settles. Contact sports, swimming and helmets that press on the ears should be avoided for at least **4–6 weeks**.
Months 3–6	The ear folds settle, the scar fades and the final appearance becomes apparent. Mild numbness of the ears usually improves during this period.

8 Possible risks and complications

As with any medical procedure, this procedure carries some risks. Below, **expected temporary effects** and **complications** are listed separately and grouped by how often they occur. The figures given are **approximate** values based on the medical literature; your personal risk may be lower or higher depending on your age, general health, smoking and previous operations.

The great majority of complications are **temporary** and improve with appropriate treatment. Very rare but serious events are also included; being on this list does not mean they will happen to you, but you need to know about them to make an informed decision.

Expected, temporary effects (not complications — a normal part of healing)

- Pain, throbbing and a feeling of tightness in the first few days
- Swelling and bruising of the ears (1–2 weeks)
- Temporary numbness of the ear skin and sensitivity to cold (weeks to months)
- A fine scar behind the ear that fades over time

Common conditions (most are temporary or resolve with treatment)

- **Stitches that can be felt or come through the skin** — In about **5–10 out of 100** patients, the permanent stitches shaping the cartilage may be felt behind the ear months or years later, come through the skin or form a small inflamed lump (granuloma). This is usually corrected with a simple procedure to remove the stitch.
- **Partial recurrence (ear moving forward again)** — In about **5–10 out of 100** patients the ear may partly return towards its original position; a correction can be performed if needed.
- **Mild asymmetry** — The two ears are never exactly the same; differences of a few millimetres after surgery are normal. If there is a noticeable difference, it can be corrected.

Op. Dr. Murat Ayata Private Clinic
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

Uncommon conditions

- **Collection of blood in the ear (haematoma)** — Occurs in about **1–3 out of 100** patients, usually within the first 48 hours. **Severe pain that keeps increasing on one side** is its most important sign. It must be **drained urgently**, before it compromises the blood supply to the cartilage; if treatment is delayed, cartilage damage and a permanent deformity ("cauliflower ear") may develop.
- **Wound infection** — Occurs in about **1–5 out of 100** patients; it usually settles with antibiotics.
- **Thick scar or keloid** — In about **1–3 out of 100** patients, especially those with darker skin or a tendency to keloids, a thick raised scar may form behind the ear. It may need treatment with steroid injections, silicone gel or surgery, and may recur. Please tell us if you have developed a keloid before.
- **Overcorrection or unnatural appearance** — The ears may look too flat against the head, the fold may be too sharp, or the middle of the ear may sit further back than the top and bottom ("telephone ear" appearance); corrective surgery can be performed if needed.

Rare or very rare, but potentially serious conditions

- **Inflammation of the cartilage lining (perichondritis)** — Occurs in **fewer than 1 in 100** patients. It shows as increasing redness, swelling, warmth and severe pain in the ear. It may require intravenous antibiotics and, if necessary, surgical cleaning; if treatment is delayed, cartilage loss and a **permanent deformity** may result.
- **Loss of blood supply to the skin or cartilage (necrosis)** — Very rare; usually associated with an overly tight dressing, an untreated haematoma or infection. Wound care and sometimes reconstructive surgery may be needed.
- **Permanent reduced sensation or chronic pain** — Numbness of the ear skin or pain in cold weather may rarely be permanent.
- **Narrowing of the ear canal** — Very rarely, after concha reduction or fixation, the entrance to the ear canal may narrow; correction may be needed.

General risks that apply to any surgical / medical procedure

- **Anaesthesia risks:** If general anaesthesia or sedation is used, its risks (nausea, sore throat, dental damage, allergic reaction and, very rarely, heart or lung problems) will be explained separately by the anaesthetist, and your consent will be obtained on a **separate anaesthesia consent form**.
- **Bleeding and blood transfusion:** The need for a blood transfusion in these procedures is very rare. In the event of serious bleeding, blood or blood products may be needed; these carry their own very rare risks (such as allergy or infection).
- **Infection:** A wound infection after surgery is usually treated with antibiotics; rarely, drainage (letting the pus out) may be needed.
- **Blood clots in the leg veins (DVT) and in the lungs (pulmonary embolism):** Very rare in short head and neck operations. Walking early reduces the risk. Hormone medicines, smoking, obesity and a previous history of clots increase the risk.
- **Drug allergy:** An allergic reaction may develop to the medicines, antiseptics, tapes or suture materials used.
- **Scarring and healing problems:** How scars form varies from person to person; very rarely a thick (hypertrophic) scar or keloid may form.
- **Risk to life:** Death in planned head and neck surgery is extremely rare today; however, no surgical procedure is entirely free of risk. A serious complication may require treatment in intensive care and may result in permanent disability.

This list does not include every possible complication; other unforeseeable and very rare events may occur. If a complication develops, your doctor will plan the necessary treatment and follow-up; this may require additional treatment, medication, procedures or a further operation.

Op. Dr. Murat Ayata Private Clinic
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

9 When to contact us immediately

- **Severe ear pain on one side that keeps increasing** (especially in the first 48 hours) — this may be a sign of a blood collection
- Increasing blood seeping through the dressing, or the dressing becoming soaked with blood
- **Fever above 38.5 °C**, increasing redness, swelling, warmth or foul-smelling discharge from the ear
- A dressing that feels too tight, or the ear turning blue or changing colour
- A blow to the ear or a sudden change in its shape

If any of these occur, call your doctor without delay: +90 288 502 50 20 · WhatsApp +90 555 333 21 20. If you cannot reach us, go to the nearest emergency department or call 112 (the emergency number in Türkiye and across Europe).

10 Medication and everyday-life advice

- Do not get the dressing wet or remove it yourself before it is taken off at the clinic. Once it has been removed, you may shower gently from the day your doctor advises.
- Wear the headband regularly for the period you are told; this is important for a lasting result.
- In the first few weeks avoid lying on your ears, folding the ears forward, and pulling tight-necked clothes over the ears.
- Protect the ears from the sun and extreme cold for at least 3 months; because of reduced sensation, a burn or frostbite may go unnoticed.
- Supervise your child so that they do not pull at the dressing or headband.
- Take the medicines prescribed for you (antibiotics, painkillers, etc.) at the recommended dose and for the recommended time. Do not take aspirin or similar medicines that may increase bleeding without asking your doctor.
- On flights or journeys longer than 4 hours after surgery, drink plenty of water, move around regularly and wear compression stockings if advised; this reduces the risk of leg clots.
- During recovery you are advised not to drink alcohol and to avoid smoking and nicotine products.
- Please attend all follow-up appointments. If you have travelled from abroad, follow-up after your return can continue by online consultation and photos/videos; if you need to see a local doctor, you will be given a written operation report and our recommendations.

11 Unexpected findings during the procedure, tissue samples and blood products

If an unforeseen situation arises during the procedure, your doctor will stay within the scope of the procedure you have consented to on this form. **Unless there is a risk to life or of serious harm to an organ**, no additional procedure that you have not consented to will be performed; in such a case the procedure will be brought to a close at a safe point, and you will be informed afterwards so that the decision can be made together with you. If your life is in danger, the emergency treatment needed to protect your life or the organ will be carried out.

Tissues removed during the procedure will be sent for **pathological examination** where necessary to confirm the diagnosis. Additional treatment or follow-up may be needed depending on the result.

Blood transfusion is very rarely needed in these procedures and will only be given when it is vital. If you do not accept blood or blood products (for example for religious reasons), you must tell us before the procedure; the possible consequences of this decision will be discussed with you separately and recorded under “Information specific to you”.

Op. Dr. Murat Ayata Private Clinic
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

12 Photographs, image records and your personal data

Photographs/videos may be taken before and after the procedure for planning, follow-up and assessment of the result. These records form part of your medical file and are kept confidential. Your images will **never be used for advertising, promotion or social media.**

Your images may be used for any other purpose **only with your separate and explicit permission**. Declining will not affect your treatment in any way, and you may withdraw your permission at any time:

- ☐ I agree that my images are used **only for medical records and treatment follow-up**.
- ☐ I permit my images to be used for scientific publication and medical education **in a way that does not identify me** (face/eye area covered).
- ☐ I permit my images to be used for scientific publication and medical education **in a way that may identify me**.
- ☐ I **do not permit** scientific/educational use.

Your personal and health data are processed under the Turkish Personal Data Protection Law No. 6698 (KVKK) and the relevant health legislation, for the purposes of diagnosis, treatment and legal obligations, by persons bound by professional confidentiality. You may consult our Data Protection Information Notice for details.

13 Information and notes specific to you (completed by the doctor)

Patient-specific additional risks (co-existing diseases, medicines, previous operations), details of the plan, the patient's expectations, questions asked and answers given. If there is nothing to add, write "none".

14 Patient declaration

- ☐ I have read and understood this form in my own language. I was given enough time to think; I had the opportunity to ask questions and received satisfactory answers.
- ☐ My condition, the aim of the recommended procedure, how it will be done, by whom and where, its approximate duration, expected benefits, likelihood of success, alternatives and what may happen if it is not done have been explained to me verbally and in writing.
- ☐ The common and potentially serious risks, and the possible consequences of these complications (including that further treatment or a repeat procedure/operation may be needed), have been explained to me. I understand that the outcome of any medical procedure **cannot be 100% guaranteed**.
- ☐ I have given accurate and complete information about my health, the medicines I take, my allergies and past illnesses. I understand that incomplete or incorrect information may increase the risks.
- ☐ I understand that following the instructions given before and after the procedure plays a part in the result, and that attending follow-up appointments is important.
- ☐ I know that I can withdraw my consent at any time before the procedure begins. I understand that signing this form does not remove the doctor's duty of care.
- ☐ I am making this decision of my own free will, without pressure from anyone.
- ☐ I understand that wearing the headband for the recommended period after surgery plays a part in the result, and that not following these instructions may increase the risk of recurrence.

Op. Dr. Murat Ayata Private Clinic
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

☐ I understand that this procedure is **cosmetic**, that it is not medically necessary and that I always have the option not to have it. I have discussed my expectations with my doctor, and the realistic result that can be achieved has been explained to me.

☐ I understand that perfect symmetry or an exact copy of the appearance in a photograph cannot be guaranteed, that the final result depends on how my tissues heal, and that in some cases a correction procedure may be needed.

☐ **I GIVE MY CONSENT to the procedure described above.**

☐ **I DO NOT ACCEPT the procedure described above. The possible consequences of refusing have been explained to me.**

We recommend the patient writes in their own handwriting: "I have read and understood this form and I give my consent."

15 Signatures

	FULL NAME	SIGNATURE	DATE / TIME
Patient			
Legal representative (parent / guardian) — relationship:			
Doctor providing information and performing the procedure			
Interpreter (if needed) — language:			
Witness (if needed)			

Interpreter's statement: I have translated this form and the doctor's verbal explanations to the patient / legal representative completely and accurately, in a language they understand.

FOR PATIENTS UNDER 18 OR WITH LIMITED DECISION-MAKING CAPACITY

For patients under 18, consent is given by a **parent or legal guardian**. The child is informed in a way appropriate to their age and maturity and their view is sought. For patients lacking capacity, consent is given by the legal representative.

☐ If we share parental responsibility, I declare that the other parent is aware of and agrees to this procedure.

☐ I declare that there is no court order or dispute concerning custody / guardianship (if there is, the relevant document has been shown to the doctor).

The child was informed in an age-appropriate way; the child's view: _____

This form has been prepared in two copies: one is kept in the patient's file and one is given to the patient / legal representative.

Op. Dr. Murat Ayata Private Clinic
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

WITHDRAWAL OF CONSENT (COMPLETE ONLY IF YOU CHANGE YOUR MIND)

I withdraw the consent I gave above. The possible consequences of withdrawing have been explained to me.

Name, signature, date and time of patient/representative withdrawing consent: _____

This form has been prepared in accordance with the Turkish Patient Rights Regulation and the informed consent principles of the Turkish Medical Association. It complements, but does not replace, the verbal explanation given by your doctor.